

Trial	BEST (N=131)	BASICS (N=300)	BAOCHE (N=217)	ATTENTION (N=340)
Population	Basilar or V4 vert artery (9%) occlusion, <u>LNW ≤ 8h</u> , China	Basilar occlusion, <u>LNW ≤ 6h</u> , 7 countries	Basilar occlusion, <u>LNW 6-24 h</u> , age <u>≤80 y</u> , <u>NIHSS ≥6</u> , <u>PC-ASPECTS ≥6</u> , China	Basilar occlusion, <u>LNW ≤ 12h</u> , <u>NIHSS ≥10</u> , see note*, China
Median NIHSS (intervention / control)	32/26	21/22	20/19	24/24
IV-tPA (intervention / control)	27%/32%	78.6%/79.5%	14%/21%	31%/34% *urokinase in minority
Intervention / Control	1:1 EVT + medical tx vs. medical tx alone (ATTENTION → 2:1 randomization) *IV-tPA if eligible			
Co-intervention(s) in EVT arm	5% IA-tPA alone, 4% angioplasty alone, 26% stent placement		55% intracranial angioplasty/stenting after failed thrombectomy	40% intracranial angioplasty/stenting (8% extracranial angioplasty/stenting)
1° Outcome	% mRS 0-3 @ 3m			
Tx Effect (ITT)	EVT: 42%, Medical: 32%; adjusted odds ratio 1.74 (0.81-3.74) *as-treated → aOR 3.02(1.31-7.00)	EVT: 44.2%, Medical: 37.7%; risk ratio 1.18 (0.92-1.50) *as-treated → similar results	EVT: 46%, Medical: 24%; rate ratio 1.81 (1.26-2.60) * original 1° outcome → mRS 0-4 @ 3m: aRR 1.21 (0.95-1.54)	EVT: 46%, Medical: 23%; rate ratio 2.06 (1.46-2.91)
2° Outcomes	No diff in mortality @ 3m	No diff in mRS 0-2 or 0-4 @ 3m, 24h NIHSS, or mortality @ 3m	EVT → mRS 0-2 @ 3m: RR 2.75 (39% vs 14%), Dramatic neurologic improvement @ 24h: RR 2.50 (25% vs 10%), basilar artery patency @ 24h: RR 4.53 (92% vs 19%), mortality @ 3m non-significantly lower (RR 0.75)	EVT → mRS 0-2 @ 3m: 3.17 (33% vs 11%), NIHSS lower post-EVT, basilar artery patency @ 24-72h: RR 2.58 (91% vs 38%), mortality @ 3m significantly lower (RR 0.66)
Sx ICH (EVT arm)	8%	4.5%	9%	5%
Procedural complications (EVT)		3.2%	11%	14%
Limitations	Terminated early (target N=344), small sample size, high crossover rate, poor recruitment	Powered for 16% diff in % mRS 0-3 @ 3m b/w trial arms, recruitment	Terminated early (prespecified interim analysis → superiority, target N=318), 27% women, predominantly atherosclerotic stroke	44% atherosclerotic stroke

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BASICS crossover: 3/154 assigned EVT received medical treatment only, and 7/146 assigned medical treatment received EVT. The 29.2% figure concerns eligible patients treated outside the trial. Subgroup findings were descriptive, underpowered and not multiplicity-adjusted; the proximal-basilar interval crossed 1.

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dspace.library.uu.nl/server/api/core/bitstreams/bf7f2173-184f-4af6-8e45-be6d8277a547/content

ATTENTION historical trial selection: age <80 years required pc-ASPECTS ≥6 and prestroke mRS 0-2; age ≥80 years required pc-ASPECTS ≥8 and prestroke mRS 0. The trial also required NIHSS ≥10 and randomization within 12 hours of estimated basilar-artery occlusion. These historical entry criteria are not current stand-alone EVT eligibility rules.

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www.binasss.sa.cr/oct22/53.pdf

Subgroup Analyses	Interaction b/w tx and occlusion site → vert occlusion (small N) better w/ medical tx and basilar occlusion better w/ intervention tx (type 1 error?)		More robust EVT tx effect in stratified analyses for younger and lower NIHSS subgroups	More robust EVT tx effect in stratified analyses for younger, higher NIHSS, more proximal basilar occlusion, non-ICAD, and cryptogenic stroke etiology subgroups Tx effect similar in <6h treated and ≥6h treated subgroups
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